

# ICU Documentation Capture — Quick Card

FY2026 | CC/MCC per CMS Appendix C | red = MCC (highest \$) orange = CC grey = no severity weight

Educational; not official CMS/AMA. Verify before billing.

## CRITICAL CARE TIME (99291 / 99292)

| Time / day | Code(s)              |
|------------|----------------------|
| 30–74 min  | 99291                |
| 75–104     | 99291 + 99292 x1     |
| 105–134    | 99291 + 99292 x2     |
| 135–164    | 99291 + 99292 x3     |
| <30 min    | not CC → 99231–99233 |

99291 = 4.50 wRVU (~\$199); 99292 = 2.25 wRVU (~\$100). Count bedside + unit time on that patient; aggregate over the day.

## PROCEDURES — get it paid

- Carve procedure minutes OUT of critical care time.
- Append **-25** to the E&M; (99291) when done same day as a procedure.
- 59 / X{EeSsPpUu}** for distinct bundled procedures; **-76/-77** repeat.
- Note: indication, consent, technique/site, findings, devices placed + confirmation (CXR), complications (or none), attestation.
- Bundled into CC (never separate): lines/IV access, NG/lavage, CXR read, pulse ox, vent mgmt, cardiac output, transcutaneous pacing.

## \$ ANCHORS (national, FY2026)

- Capturing one **MCC** ≈ **+\$3,000–\$6,000**; one **CC** ≈ +\$1,200–\$3,000.
- Tier set by the SINGLE highest secondary dx — 1st MCC/CC is the valuable one; extras add \$0.
- Vent >96h vs ≤96h** = DRG 207 (~\$43.5K) vs 208 (~\$18.6K): biggest single lever — document continuous hours.
- Highest-weight ICU DRGs: trach+vent>96h (003/004), sepsis w/ vent>96h (870), acute leukemia (834).

## TOP MCC CAPTURES (highest value)

**J96.0x / J96.2x** Acute (or acute-on-chronic) resp failure  
**J80** ARDS  
**R65.20 / .21** Severe sepsis / septic shock  
**A41.x** Sepsis (name organism)  
**R57.0 / R57.1** Cardiogenic / hypovolemic shock  
**I50.21/.23/.31/.33** Acute / acute-on-chronic HF  
**I21.x** Acute MI (STEMI/NSTEMI)  
**I46.x / R09.2 / I49.01** Cardiac arrest / resp arrest / VF  
**I71.0x / I33.0 / I40.x** Aortic dissection / endocarditis / acute myocarditis  
**G93.41 / G92.8** Metabolic / toxic encephalopathy  
**R40.2x / G93.82 / G93.6** Coma / brain death / cerebral edema  
**I61 / I60 / S06(A)** ICH / SAH / acute TBI  
**E43 / E41** Severe malnutrition / marasmus  
**K85.x** Acute pancreatitis (incl. K85.90)  
**I85.01 / K25.0/.4** Variceal / ulcer bleed  
**K72.x / K65.2 / K76.7** Hepatic failure / SBP / hepatorenal  
**K55.069 / K65.0** Mesenteric infarction / peritonitis  
**E10.1x/E11.1x / E11.0x** DKA / HHS  
**E05.91 / E03.5 / E27.2** Thyroid storm / myxedema / adrenal crisis  
**C92.00 / C91.00** Acute leukemia, not in remission  
**D65 / E88.3 / M31.10** DIC / tumor lysis / TTP-TMA  
**D57.0x** Sickle-cell crisis  
**B37.7 / M72.6** Candidal sepsis / necrotizing fasciitis  
**L89 stg 3/4/unstageable** Pressure injury — document POA!  
**J93.0** Spontaneous tension pneumothorax

## TOP CC CAPTURES (common, often missed)

**N17.x** AKI / ATN (CC, not MCC)  
**E87.20/.21** Acidosis incl. lactic (if not integral)  
**E44.0** Moderate malnutrition  
**J96.1x** Chronic respiratory failure  
**I50.x2** Chronic HF  
**J44.1 / J44.0** COPD exacerbation / with infection  
**J69.0 / J15.x** Aspiration / GN-MRSA pneumonia  
**K92.x** GI hemorrhage / melena / hematemesis  
**K70.3x / K74.6x** Cirrhosis (± ascites)  
**F05 / G93.40** Delirium / encephalopathy unspecified  
**G72.81 / G62.81** ICU-acquired weakness (myo/neuropathy)  
**I31.4 / I47.2 / I16.1** Tamponade / VT / HTN emergency  
**I21.A1 / I26.99** Type-2 MI / PE w/o cor pulmonale  
**I63.x / G61.0** Ischemic stroke / Guillain-Barré  
**D62 / D69.3 / D61.81x** Blood-loss anemia / ITP / pancytopenia  
**M62.82 / D68.4** Rhabdomyolysis / acquired coagulopathy  
**E87.0 / E87.1** Hyper- / hyponatremia  
**E27.1 / E15 / R64** Adrenal insuff. / hypoglycemic coma / cachexia  
**T80.211A / T83.511A** CLABSI / CAUTI (POA!)  
**J95.851 / J95.02** VAP / trach stoma infection

## DON'T ASSUME IT COUNTS (NO weight)

- Hepatic encephalopathy K76.82 • Neutropenia D70.x • HIT D75.82
- K+/K- E87.5/.6 • Ca++ E83.52 • Mg E83.42 • dehydration E86.x
- Atrial fibrillation I48.x • bacteremia R78.81 • SIRS
- Pulm HTN I27.2x, cor pulmonale I27.81, Eisenmenger (only I27.0/.1 = CC)
- Dysphagia R13.1x • deconditioning M62.81 • BMI Z68
- Pleural effusion J90 • ascites R18.8 • MRSA infection A49.02
- Pressure injury stage 1/2 • dialysis dependence Z99.2

## SPECIFICITY RULES

- Acute vs chronic vs acute-on-chronic (resp failure, HF, renal).
- Resp failure: hypoxic vs hypercapnic.
- Sepsis → link each organ dysfunction; name organism.
- AKI: hepatorenal (K76.7, MCC) vs ATN/prerenal (N17, CC).
- Malnutrition: physician dx + ASPEN/GLIM criteria + severity.
- Document POA for pressure injury, CLABSI, CAUTI (else HAC).